Preparing for First Aid Emergencies

From R. Rehberg, *Sport First Aid*, 6th ed. (Champaign, IL: Human Kinetics, 2026), 13-26.

To prepare your team for competition, you plan practices, develop playing strategies, and train your athletes. This pregame planning process is essential to your team's success. The same is true in sport first aid; to handle injuries effectively, you must plan for them. You don't want to be caught unprepared for a critical health situation involving blood loss, unresponsiveness, or breathing difficulties. This chapter shows you how to prepare for urgent situations by enhancing and maintaining your education, gathering athletes' health records, stocking your first aid kit, incorporating physical screenings and conditioning into your overall program, and developing emergency plans.

Your Sport First Aid Education

HK Coach Education strongly recommends that you supplement what you learn from this book (and from the Sport First Aid course) with certification in cardiopulmonary resuscitation (CPR) and automated external defibrillator (AED) use. You can obtain certification through the American Red Cross or the American Heart Association. These programs are recognized nationally as standards for providing first aid care. Upon achieving certification, you will be expected to provide the standard of care taught in the certification program.

Many youth and interscholastic sport organizations may also require additional education in the following areas:

- Concussions
- Heat illness, hydration, and environmental factors
- Cardiac emergencies

Keeping Current

The field of sports medicine is constantly improving. Therefore, you need to keep current on the latest developments in sport first aid. The sport first aid techniques used in the future will be different from and better than the methods advocated now. Following are ways to keep up with these changes:

- Read current sports medicine books, and take appropriate online education courses to maintain certifications and learn the newest techniques.
- Keep your first aid training and CPR certification current. Most CPR certifications only last for two years.

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- Attend sports medicine and sport first aid seminars and clinics. The Sport First Aid course will be updated as advances in this area warrant, so stay current by attending the course every few years.

Recognizing Limitations

Even if you educate yourself extensively in sport first aid, do not attempt the duties of a physician, athletic trainer, or any other licensed athletic health care provider. Recognize your limitations, and provide only the care that you are qualified to provide. You can cause many years of damage to an athlete by overstepping the limits of your training. In addition, if you act irresponsibly and harm an athlete, you may end up the target of a lawsuit. If medical personnel are present, give them complete control to handle any illnesses or injuries. You can assist them if requested.

Keeping Athletes' Health Records

Like most coaches, you probably keep statistical records of your athletes' performances. To become familiar with each athlete's health information, collect the following information:

- Consent form
- Health history form
- Emergency information card

Consent Form

Remember, you cannot give first aid care to a minor unless you have consent. Before the season, you must have parents or legal guardians complete and return an explicitly worded consent form for their children. A consent form informs the athlete and the parent or guardian of the inherent risks of sport, and it requests permission from the parent or guardian to treat the child for an emergency illness or injury. A sample consent form is shown in figure 2.1.

Health History Form

To train your athletes safely, it is essential that you know of any health conditions that could affect their participation in sports. Diabetes, asthma, epilepsy, heart murmurs, and skin conditions are among these conditions. If a health care provider clears an athlete with a health condition for participation, you should have a record indicating the athlete's health concerns and any related medications they need to take. To collect this kind of information, issue a health history form such as the one shown in figure 2.2.

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Emergency Information Card

In the event of an emergency, you must be able to contact the athlete's health care provider, parent or guardian (if the athlete is a minor), and a secondary contact. An emergency information card (see figure 2.3) indicates names and numbers of emergency contacts as well as medical insurance information. It should also alert you to information about preexisting medical conditions that may influence the treatment of an athlete. You must have a completed card from each athlete before the season begins. When the team practices or competes away from your school or other organization, you should take copies of the team's cards with you.

A person's health history and injury status are confidential pieces of information. Therefore, respect each athlete's privacy by keeping this information in a secure location. In addition, do not speak publicly (e.g., to fans, other athletes, media) about an athlete's condition unless you have written permission from the athlete (or their parent or guardian if a minor).

Checking Facilities and Equipment

Although preparation and care of the playing area may be the responsibility of a groundskeeper or janitor, you are responsible for checking its safety. Litter, slippery floors, broken goals, worn playing surfaces, and the like can lead to injury. Check for hazards, and have them fixed before the season begins. Figure 2.4 provides a sample facilities inspection checklist. Use this sample to develop a checklist specific to your facilities.

Check sports equipment before every season begins. Inspect sticks, rackets, bats, gymnastics apparatus, protective helmets and pads, and other equipment for damage. Be sure that goalposts, net standards, landing pits, and gymnastics apparatus are well padded and secured. This could be the job of the equipment manager if your school or organization has one.

You will also need to prepare equipment and supplies for handling injuries. A first aid kit and ice cooler must be available on the sidelines at every practice and competition. When stocking your first aid kit, include only the items necessary for administering basic sport first aid. Omit all medicines, including over-the-counter (such as aspirin, pain medications, or decongestants) and prescription drugs. The exception to this is if a student has a prescribed medication and you have written clearance that you can administer it if needed (e.g., insulin, EpiPen, asthma medication). Best practice with regard to medication is if over-the-counter medication is needed, written clearance from the doctor, parent, or guardian should be provided if the student is a minor (under 18 years old) before administering medication. With any medication questions or concerns,

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it is important to check with your state and local laws, rules, and regulations. Omit items that may cause allergic reactions, such as solutions made with iodine and gloves made of latex. Be aware of latex allergies, as many first aid products may contain latex, such as tapes and wraps.

Stocking the First Aid Kit

A well-stocked first aid kit includes the following items:

- Antibacterial soap or wipes
- Arm sling
- Athletic tape—one-and-a-half inch
- Bandage scissors
- Bandage strips—assorted sizes
- Blood spill kit
- Cell phone
- Contact lens case
- Cotton swabs
- CPR mouth barrier or pocket mask
- Elastic wraps—three inch, four inch, and six inch
- Emergency blanket
- Examination gloves—latex free
- Eye patch
- First aid cream or antibacterial ointment
- Foam rubber—one-eighth inch, one-fourth inch, and one-half inch
- Hand sanitizer
- Insect sting kit
- Instant chemical cold packs or bags (to hold ice for ice packs)
- List of emergency phone numbers
- Medical release consent forms
- Mirror
- Moleskin
- Nail clippers
- Oral thermometer (to determine if an athlete has a fever due to illness)
- Penlight
- Petroleum jelly
- Plastic bags for crushed ice
- Prewrap—underwrap for tape (for taping)
- Rectal thermometer (for use in cases of suspected heat illness)
- Rescue blanket
- Safety glasses—for first aiders
- Safety pins
- Saline solution for eyes
- Sterile gauze pads—three-inch and four-inch squares (preferably nonstick)
- Sterile gauze rolls
- Sunscreen—sun protection factor (SPF) 30 or greater
- Tape adherent and tape remover
- Tongue depressors
- Tooth-saver kit
- Triangular bandages
- Tweezers

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Preparing Athletes to Perform

Athletes who are in poor physical condition are more prone to injury. To ensure that athletes are ready for practice and competition, establish these routines:

- Preseason physical exam
- Preseason screening
- Preseason conditioning
- Proper warm-up and cool-down
- Use of protective equipment, bracing, and taping
- Correct skill instruction
- Sound nutritional guidance
- Ban on rough or boisterous play

Preseason Physical Exam

The first step in preparing an athlete for participation in sports is to require a preseason physical exam. For this exam, the athlete's health care provider conducts a general health examination as well as circulatory, respiratory, neurological, orthopedic, vision, and hearing tests. The exam also includes blood and urine analyses. The health care provider should note and consider any preexisting or potential health conditions when deciding if the athlete has medical clearance for participation.

All athletes must turn in their physical exam forms before the season begins. Familiarize yourself with the records of athletes who have specific conditions that could affect their participation, such as asthma, diabetes, severe allergies, and epilepsy. Keep all forms in a secure file for future reference.

Preseason Screening

During the preseason screening, the coach can identify players who are most qualified for specific positions, as well as those who may be developed to play the position. This includes identifying athletes who need additional development and conditioning.

Although the physical exam detects specific health problems, it does not provide insight about an athlete's fitness. A preseason screening can provide this information. Preseason screenings should be conducted in the off-season by a specially trained health professional, such as an athletic trainer, or by a fitness professional, such as a certified strength and conditioning coach. Depending on the sport, each athlete should be evaluated for the following:

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Limiting or Disqualifying Medical Conditions

The examining health care provider may discover a condition that limits an athlete's participation or disqualifies them from competition. Common disqualifying or limiting conditions may include the following:

- Uncontrolled diabetes
- Uncontrolled asthma
- Heart conditions
- Uncontrolled high blood pressure
- Epilepsy
- Previous head injury
- Previous spinal injury
- Chronic orthopedic problem (e.g., unstable knees, ankles, or shoulders)

In the past, individuals with the sickle-cell trait were often disqualified from athletic participation because it was believed that carrying the trait put them at particular risk for sudden death. However, in a large-scale study of military recruits, Casa and colleagues (2012) showed that the risk of sudden death was actually higher in those without the sickle-cell trait than in those with the sickle-cell trait. This finding does not mean that carrying the sickle-cell trait is without risk in sports; it means that this condition should not disqualify athletes from participation in sports.

- *Strength* in the muscle groups most often used in the particular sport (e.g., a football player's neck strength or a basketball player's ankle strength)
- *Flexibility* or tightness in the major muscle groups and tendons (hamstrings, quadriceps, shoulder muscles, calf muscles, and Achilles tendons)
- *Endurance* in muscles that undergo repetitive or sustained contraction
- *Cardiovascular endurance* (especially for endurance athletes such as cross country runners, track athletes, triathletes, and cyclists)
- *Body composition* or percent body fat (especially important for wrestlers, gymnasts, and track athletes who severely restrict their diets to control their weight)
- *Coordination in the upper and lower body* to determine if an athlete's muscles fire quickly enough to protect a joint from injury (e.g., a timed one-leg balance test to see how long they can stand on one leg without wobbling or putting the other foot down)

These tests pinpoint potential fitness problems that can lead to injury. Coaches or athletic trainers should teach athletes conditioning exercises to help them improve these problems before the season.

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Preseason Conditioning

Get athletes in shape by starting them on a conditioning program at least 6 weeks before the season. Conditioning exercises should focus on muscle strength, cardiovascular endurance, flexibility, power, and speed needed for the sport.

To improve strength, consider these basic guidelines: Athletes need to perform at least 2 sets of 6 to 8 repetitions of each exercise, 3 days a week. Postpubescent athletes should lift at least 70 percent of their maximum weight to gain strength. Although well-supervised resistance training has been proven safe for prepubescent athletes, you can avoid weightlifting-related injuries by emphasizing activities that require these athletes to support their own body weight (e.g., push-ups). Training 3 days a week for at least 20 continuous minutes is necessary to improve cardiovascular endurance. To improve flexibility, athletes should perform stretching exercises at least 5 days a week.

For more information on training and conditioning in the preseason, consult *Successful Coaching* (Martens and Vealey 2024).

Proper Warm-Up and Cool-Down

Be sure that your athletes warm up before workouts, practices, and competitions. Warming up doesn't mean going out 5 minutes before practice and hitting or throwing a few balls. A proper warm-up is an exercise routine that prepares the body for vigorous physical activity. Athletes should warm up at least 15 minutes prior to activity using this sequence:

- *General warm-up.* Athletes jog or bike at a low intensity for 5 to 10 minutes. The intensity of the general warm-up should cause a slight increase in the heart and breathing rates. It should also cause the athlete to break into a light sweat. The general warm-up helps to prepare the heart, lungs, muscles, and tendons for vigorous activity. Ultimately, it helps to prevent injury and to improve performance.
- *Light exercises.* After a general warm-up, athletes continue to warm up specific areas with exercises such as push-ups, jumping jacks, abdominal crunches, lunges, or carioca (cross-over run).
- *Sport-specific drills.* These drills allow athletes to practice the skills of their particular sport. For example, sport-specific softball drills include batting and throwing. In tennis and racquetball drills, athletes practice serves as well as backhand and forehand shots.

At the end of each practice, workout, or competition, athletes should gradually cool their bodies down. In other words, they should slowly reduce the intensity of their activity until their heart and breathing rates drop to near normal resting levels. Suddenly

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stopping exercise inhibits recovery from activity and can lead to problems such as fainting. Cool-down activities may include walking or light jogging for 5 to 10 minutes.

Athletes should conclude the cool-down with stretching. The muscles are very warm after activity, so they will stretch more easily and maintain the stretched position longer. Therefore, the cool-down period is a prime time for athletes to achieve long-term improvements in their flexibility. In general, muscles need to be stretched for a total of 2 to 3 minutes a day to obtain lasting improvements in length. Athletes should stretch each muscle group (appropriate to their sport) in 15- to 30-second repetitions, for a total of 2 to 3 minutes.

Protective Equipment, Bracing, and Taping

As a coach, you should become an expert on how to properly fit and use your sport's protective equipment and instruct athletes how to correctly fit and wear it. Conduct surprise inspections to ensure that athletes adequately maintain and fit their equipment. Proper fit is especially important in contact sports such as American football, where helmets must be specially fitted and the athletes are required to wear all of their protective pads. To minimize the risk of broken equipment, conduct routine inspections and have shoulder pads and helmets refurbished and retested on a regular basis. An equipment manager would be ideal for this role if available.

Two important and often neglected pieces of equipment are mouth guards and safety glasses or goggles. Mouth guards help prevent sport-related dental injuries in a variety of sports. If any chance of eye injury exists in a sport, particularly in a contact or racket sport, athletes should use protective eyewear.

Some coaches think that bracing and taping can help athletes prevent injuries. For example, a coach might require football players to wear knee braces to prevent knee injuries or basketball players to tape their ankles to prevent ankle sprains. The use of taping and bracing is common in rehabilitation of athletic injuries. However, its use for injury prevention in non-injured athletes is questionable (see sidebar To Brace or Not to Brace?).

Correct Skill Instruction

Many athletes become injured because they use unsafe technique. Since the technique of spear tackling was banned in football, the number of head and neck injuries among football players decreased. Baseball or softball players who dive headfirst into a base instead of sliding feet first are prone to tooth, head, and neck injuries. Many tennis players experience tennis elbow because they use incorrect backhand techniques.

You can help prevent these and other injuries by teaching your athletes safe and proper skills. Also, keep an eye out for athletes who use potentially harmful techniques. Warn

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them of the injuries they could suffer, and reinstruct them in the use of appropriate skills.

Sound Nutritional Guidance

NOTE: The following guidelines were updated in January 2026. See the updated USDA guidelines at https://realfood.gov.

Encourage your athletes to eat balanced meals according to the MyPlate guidelines (see figure 2.5). You can help athletes and their parents determine nutrient needs by recommending that they go to the MyPlate website (www.myplate.gov). Athletes can use tools from this website to help them determine their nutrient needs based on their size, gender, age, and activity level.

Also encourage athletes to drink plenty of fluids to maintain adequate hydration.

To Brace or Not to Brace?

Are protective bracing and taping all they're cracked up to be? They are certainly not substitutes for good technique and training. Strength, flexibility, endurance, and power are the keys to preventing injury; bracing and taping are of secondary importance. With the exception of some types of ankle braces, evidence that bracing or taping prevents injuries has not been conclusive. It is difficult to prove whether a decrease in injuries can be attributed to wearing a protective brace. Ultimately, the athlete (and parent or guardian if a minor) needs to decide on the use of preventive bracing and taping. Consulting an AT or appropriate health care provider about the benefits of taping and bracing can help you understand the right decision. Some states may require a note from a health care provider that justifies the need for the use of a brace or splint in order for the athlete to participate with it.

Eating on the Road

With planning and organization, an adequate high-carbohydrate, moderate-protein, low-fat (20% to 25% of total calories) diet can be obtained on the road. If budgets allow, bring along dried fruits, juices, low-fat granola bars, and other snacks that offer healthy alternatives to those offered in vending machines. Many restaurants honor special requests for teams, such as pasta bars, low-fat sandwich options, and fresh fruits and vegetables. In addition to paying attention to appropriate portions for adequate nutrition, encourage athletes to consume juices and skim milk products instead of soft drinks; baked, broiled, or boiled meats instead of fried meats; and plenty of carbohydrate-rich foods such as potatoes, rice, pasta, breads, bagels, fruits, and

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vegetables. Digestion takes between 2 and 5 hours depending on what is eaten, so plan meals to allow adequate time for digestion before a competition.

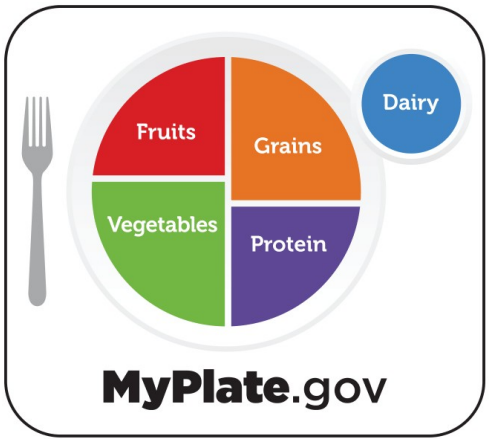


Figure 2.5 *MyPlate.*

U.S. Department of Agriculture.
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An adequate diet for an athlete is like fuel for an automobile; it is energy for performance. To better inform your athletes about the quality of fuel in their diets, and to learn more about sports drinks and other nutritional issues, read *Nancy Clark's Sports Nutrition Guidebook, Sixth Edition* (Clark 2020).

Eating for Performance

The following pregame meal guidelines can help prevent upset stomach during competition:

- Eat at least 3 hours before practices, workouts, and competitions.
- Avoid foods that are high in fat, such as french fries, potato chips, and peanut butter.
- Avoid foods that are high in fiber, such as lettuce, beans, cabbage, spinach, and nuts.
- Avoid foods that are high in sugar, such as candy bars, cakes, doughnuts, and honey.
- Eat plenty of high-carbohydrate foods that are easily digested, such as pasta, breads, low-fiber cereals, fruit juices, potatoes, and bananas.
- Eat foods that are familiar; the pregame meal is no time to try new foods.

Ban on Horseplay

Although joking around can be harmless, rough or boisterous play such as wrestling, pushing, and hitting can lead to unintended injuries. Establish a *no horseplay* rule at the start of the season, and enforce it at all times.

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Developing an Emergency Action Plan (EAP)

Emergency medical situations may arise at any time during an activity or sport event, so the sport first aid team must be prepared. In order to provide optimal care to sports participants in an emergency or life-threatening situation, expedient action is required. Developing and implementing an emergency action plan (EAP) ensures that you are prepared to respond when a participant suddenly needs your help and that they receive the best possible care.

Effective management of emergencies requires proper planning. Preparation is the key to ensuring that appropriate resources and procedures exist. Preparation should begin well in advance of the sport season, and it should be dynamic in nature. Athletic organizations have a duty to develop an EAP that can be implemented immediately when necessary and to provide appropriate standards of emergency care to all sports participants.

Formulating an EAP specific to sports has many prerequisites, including proper education and training of individuals, development of appropriate first aid procedures for events, maintenance of appropriate medical emergency equipment and supplies, and utilization of appropriate medical emergency personnel. Through careful preparticipation physical screenings, adequate medical coverage, safe practice and training techniques, and other safety avenues, some potential emergencies may be averted. However, accidents and injuries are inherent to participation in sports, and proper preparation on the part of the sport first aid team should enable each emergency to be managed appropriately.

Developing the medical EAP must take many components into account, such as what personnel will be involved, what rules will be followed, what equipment will be available, where the arena for the contest or practice will be located, when the plan will be rehearsed, how the plan will be evaluated, and what the education processes will be for the stakeholders of the EAP. The person in charge of developing the medical EAP should ensure that all involved parties are part of the planning process. The EAP must provide detailed instructions for who will act, what actions should be taken, and how and where they will be taken.

The medical EAP should be readily available to all the members of the athletic health care team at each athletic venue. When visiting teams are coming to a venue, they, too, should be provided with a copy of the medical EAP. A separate venue-specific EAP should be developed for each venue, complete with detailed instructions and information including the address of the venue, a description of the location of emergency equipment, communication methods (cell phone, radios, landline telephones), a list of

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emergency telephones, a list of emergency hand signals (for use on the field), and detailed instructions for staff and emergency care personnel.

A response plan should lead the athletic health care team through conducting a thorough evaluation of an injured athlete, activating the emergency medical system (EMS), and providing effective first aid. To get started on creating your EAP, use the following guide (adapted from American Safety & Health Institute 2015):

- *Assess*—How do I evaluate the scene and the injured athlete?
- *Alert*—How do I activate the EMS?
- *Attend*—How will first aid care be provided?

Assess

First, your plan needs to specify how you will evaluate an injured athlete. This plan should address issues such as

- what to do first when you arrive at an injured athlete's side,
- how to evaluate the safety at the scene for the injured athlete, and
- steps for evaluating responsive and unresponsive athletes.

Alert

Next, your plan should indicate how to activate the EMS. If medical personnel are not present, how do you send for medical assistance while evaluating and providing first aid care to an athlete? To help the response go more smoothly in the event of a health emergency, you should develop a plan, before the season begins, for activating the EMS. Here's an example of an effective step-by-step approach:

- *Delegate the responsibility of seeking medical help.* Choose a person who is calm and responsible. It can be an assistant coach, a parent, or an athlete. Make sure that this person is available during every practice and game.
- *Make a list of emergency phone numbers.* Save a digital copy of the list in your mobile phone, and put a printed copy in your first aid kit to take to every practice and game. Include the following phone numbers:
    - Rescue unit
    - Hospital
    - Team physician (if applicable)
    - Police
    - Fire department

  Before traveling to an away game, talk to the host coaches about emergency medical services.

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- *Take each athlete's emergency information card to every practice and game.* This information is especially important if an athlete is unresponsive and unable to tell you who you should contact or to give you that person's phone number.
- *Give an emergency response card to the contact person calling for emergency assistance.* It will prompt the caller to provide critical information to the emergency care staff. It will also help calm the caller by providing everything they need to communicate to emergency personnel. Figure 2.6 provides a sample emergency response card.
- *Complete an injury report, and keep it on file for any injury that occurs.* This form should provide the information requested in the sample shown in figure 2.7.

Attend

Finally, your plan needs to indicate how first aid care will be provided. If medical personnel are present at the time of the injury, assist them as needed while they assume the care of the injured athlete. If medical personnel are not present, provide first aid care to the extent of your qualifications. Chapters 4, 5, and 6 cover first aid basics and the proper way to move an injured athlete. Part II discusses care of specific injuries.

Handling Different Types of Injuries

Many injuries don't require emergency medical attention. An athlete who slightly twists an ankle or gets a minor bruise is not in serious condition. However, some noncritical injuries or illnesses can severely impair performance. Therefore, you should evaluate and monitor these injuries closely to ensure that no further complications occur.

All coaches should be aware of the most common serious emergencies for which a quick and effective response is critical, namely, cardiac and weather-related emergencies. You are advised to practice responding to these situations once per season as well as make sure all current guidelines are met, everyone on the sport first aid team knows their role, and their questions and concerns are addressed.

Handling Minor Injuries

For minor injuries, follow these steps:

1. Assess the injury.
2. Attend to the injury (first aid).
3. Remove the athlete from participation if the athlete is in a great deal of pain or experiences loss of function (e.g., can't walk, run, jump, or throw).
4. Contact the athlete's parent or guardian (if a minor) to discuss the injury.
5. Suggest that the athlete see a health care provider to rule out a serious injury.
6. Complete an injury report form while the incident is still fresh in your mind.

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Handling Serious Injuries

If a serious injury or illness occurs, follow these steps:

1. Assess the safety of the scene and the athlete's level of responsiveness.
2. Send a contact person to alert emergency medical personnel and the athlete's parent or guardian (if a minor).
3. Send someone to wait for the rescue team, help them open doors and gates, and direct them to the injured athlete.
4. Assess the injury.
5. Attend to the injury (first aid).
6. Assist emergency medical personnel in preparing the athlete for transportation to a medical facility.
7. Appoint someone to go with the athlete (if none of the athlete's emergency contacts are present). This person should be responsible, calm, and familiar with the athlete. Assistant coaches or parents are best for this job.
8. Complete an injury report form while the incident is still fresh in your mind.

Cardiac Emergencies

Sudden cardiac arrest is the leading cause of death in young athletes. Although the likelihood of such emergencies increases with age (spectators are more likely to need medical assistance related to heart problems), even healthy, seemingly low-risk youth can experience a cardiac event.

It is imperative that all members of the athletic health care team stay current with all appropriate national safety guidelines for sports. Staying current with these guidelines (among other important skills) will help members of the athletic health care team understand when to use an automated external defibrillator (AED) and give the athlete the best chance of a positive outcome. An AED is a medical device that analyzes heart rhythm and delivers an electrical shock (defibrillation) to help the heart reestablish an effective rhythm.

To improve likelihood of survival in sudden cardiac arrest, quickly applying an AED is of utmost importance. Although ambulances are equipped with AEDs, athletic health care team members should ensure that an AED is accessible during all athletic events and that every member knows where it is located. According to the American Heart Association, chances of survival from cardiac arrest decrease 7 to 10 percent for every minute a shock from a defibrillator is delayed for a victim with a shockable rhythm (Ibrahim 2007).

Take the time to review the EAP for your team's facility (or upon arrival at another facility), locate the AED, confirm it is operational and accessible, and visualize the steps

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necessary in case of an emergency. Taking these steps could save valuable time in the event of an emergency; it could even mean saving a life. Some states do require a separate cardiac EAP; refer to your local and state laws regarding this.

Heat Illness Emergencies

Heat illness emergencies can result from training in extreme heat and humidity, which can be deadly for athletes. Heat illness emergencies consist of heat cramps, heat exhaustion, and heatstroke.

Heat illness can occur even when you abide by your organization's rules regarding temperature and humidity limits and when you follow policies and procedures for practice limitations and restrictions. Heat illness is progressive; a person can transition from heat cramps to heatstroke very quickly. If an athlete is in the stages of heat cramps or heat exhaustion, cooling steps (see chapter 11) should be taken to help prevent them from transitioning to the much more serious condition of heatstroke. Symptoms of heatstroke include rapid heart rate, altered mental state, loss of consciousness, lack of sweating, and red skin. Heatstroke is a medical emergency; cooling an overheated athlete rapidly (within 30 minutes) can mean the difference between life and death. Using a cold-water tub or equivalent technique can help bring an athlete's body temperature to a safe level within 15 minutes.

Cold Weather Emergencies

In addition to increasing chances of other injuries because of icy, wet, or windy conditions, cold weather can cause specific cold injuries such as hypothermia, frostnip, and frostbite. Preparing for these emergencies includes monitoring the temperature and making appropriate modifications.

Lightning Emergencies

Lightning during sport events is extremely dangerous due to the exposure of the athletes on the field. The presence of lightning should be cause for immediate cessation of all outdoor activities until the storm has passed.

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FIGURE 2.1

Informed Consent Form

I hereby give my permission for _____ to participate in _____ during the athletic season beginning _____. Further, I authorize the school to provide emergency treatment of any injury or illness my child may experience if qualified medical personnel consider treatment necessary and perform the treatment. This authorization is granted only if I cannot be reached and a reasonable effort has been made to do so.

Date _____ Parent or guardian _____

Address _____

Primary phone _____ Phone (other) _____

Family physician _____ Phone _____

Medical conditions (e.g., allergies or chronic illnesses)

_____

_____

Other person to contact in case of emergency

_____

Relationship to person _____ Phone _____

My child and I are aware that participating in _____ is a potentially hazardous activity. We assume all risks associated with participation in this sport, including falls, contact with other participants, weather effects, traffic, and other reasonable risk conditions associated with the sport. All such risks to my child are known and appreciated by my child and me.

We understand and agree to the conditions outlined on this form.

Child's signature Date _____

_____

Parent's or guardian's signature Date _____

_____

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FIGURE 2.2

Preparticipation Physical Evaluation History Form

This form should be placed into the athlete's medical file and should not be shared with schools or sports organizations. The Medical Eligibility Form is the only form that should be submitted to a school or sports organization.

Disclaimer: Athletes who have a current Preparticipation Physical Evaluation (per state and local guidance) on file should not need to complete another History Form.

PREPARTICIPATION PHYSICAL EVALUATION (Interim Guidance)

HISTORY FORM

Note: Complete and sign this form (with your parents if younger than 18) before your appointment.

Name: _____ Date of birth: _____

Date of examination: _____ Sport(s): _____

Sex assigned at birth (F, M, or intersex): _____ How do you identify your gender? (F, M, non-binary, or another gender): _____

Have you had COVID-19? (check one): ☐ Y ☐ N

Have you been immunized for COVID-19? (check one): ☐ Y ☐ N If yes, have you had: ☐ One shot ☐ Two shots ☐ Three shots ☐ Booster date(s) _____

List past and current medical conditions.

_____

Have you ever had surgery? If yes, list all past surgical procedures.

_____

Medicines and supplements: List all current prescriptions, over-the-counter medicines, and supplements (herbal and nutritional).

_____

Do you have any allergies? If yes, please list all your allergies (ie, medicines, pollens, food, stinging insects).

_____

Patient Health Questionnaire Version 4 (PHQ-4)

Over the last 2 weeks, how often have you been bothered by any of the following problems? (Circle response.)

	Not at all	Several days	Over half the days	Nearly every day
Feeling nervous, anxious, or on edge	0	1	2	3
Not being able to stop or control worrying	0	1	2	3
Little interest or pleasure in doing things	0	1	2	3

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	Not at all	Several days	Over half the days	Nearly every day
Feeling down, depressed, or hopeless	0	1	2	3

(A sum of ≥3 is considered positive on either subscale [questions 1 and 2, or questions 3 and 4] for screening purposes.)

Explain "Yes" answers at the end of this form. Circle questions if you don't know the answer.

GENERAL QUESTIONS	Yes	No
1. Do you have any concerns that you would like to discuss with your provider?	☐	☐
2. Has a provider ever denied or restricted your participation in sports for any reason?	☐	☐
3. Do you have any ongoing medical issues or recent illness?	☐	☐

HEART HEALTH QUESTIONS ABOUT YOU	Yes	No
4. Have you ever passed out or nearly passed out during or after exercise?	☐	☐
5. Have you ever had discomfort, pain, tightness, or pressure in your chest during exercise?	☐	☐
6. Does your heart ever race, flutter in your chest, or skip beats (irregular beats) during exercise?	☐	☐
7. Has a doctor ever told you that you have any heart problems?	☐	☐
8. Has a doctor ever requested a test for your heart? For example, electrocardiography (ECG) or echocardiography.	☐	☐
9. Do you get light-headed or feel shorter of breath than your friends during exercise?	☐	☐
10. Have you ever had a seizure?	☐	☐

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HEART HEALTH QUESTIONS ABOUT YOUR FAMILY	Unsure	Yes	No
11. Has any family member or relative died of heart problems or had an unexpected or unexplained sudden death before age 35 years (including drowning or unexplained car crash)?	☐	☐	☐
12. Does anyone in your family have a genetic heart problem such as hypertrophic cardiomyopathy (HCM), Marfan syndrome, arrhythmogenic right ventricular cardiomyopathy (ARVC), long QT syndrome (LQTS), short QT syndrome (SQTS), Brugada syndrome, or catecholaminergic polymorphic ventricular tachycardia (CPVT)?	☐	☐	☐
13. Has anyone in your family had a pacemaker or an implanted defibrillator before age 35?	☐	☐	☐

BONE AND JOINT QUESTIONS	Yes	No
14. Have you ever had a stress fracture or an injury to a bone, muscle, ligament, joint, or tendon that caused you to miss a practice or game?	☐	☐
15. Do you have a bone, muscle, ligament, or joint injury that bothers you?	☐	☐

MEDICAL QUESTIONS	Yes	No
16. Do you cough, wheeze, or have difficulty breathing during or after exercise?	☐	☐
17. Are you missing a kidney, an eye, a testicle, your spleen, or any other organ?	☐	☐
18. Do you have groin or testicle pain or a painful bulge or hernia in the groin area?	☐	☐
19. Do you have any recurring skin rashes or rashes that come and go, including herpes or methicillin-resistant Staphylococcus aureus (MRSA)?	☐	☐
20. Have you had a concussion or head injury that caused confusion, a prolonged headache, or memory problems?	☐	☐

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MEDICAL QUESTIONS	Yes	No
21. Have you ever had numbness, had tingling, had weakness in your arms or legs, or been unable to move your arms or legs after being hit or falling?	☐	☐
22. Have you ever become ill while exercising in the heat?	☐	☐
23. Do you or does someone in your family have sickle cell trait or disease? [Unsure ☐]	☐	☐
24. Have you ever had or do you have any problems with your eyes or vision?	☐	☐
25. Do you worry about your weight?	☐	☐
26. Are you trying to or has anyone recommended that you gain or lose weight?	☐	☐
27. Are you on a special diet or do you avoid certain types of foods or food groups?	☐	☐
28. Have you ever had an eating disorder?	☐	☐

MENSTRUAL QUESTIONS	N/A	Yes	No
29. Have you ever had a menstrual period?	☐	☐	☐
30. How old were you when you had your first menstrual period?	☐	☐	☐
31. When was your most recent menstrual period?	☐	☐	☐
32. How many periods have you had in the past 12 months?	☐	☐	☐

Explain “Yes” answers here.

_____

_____

_____

_____

_____

I hereby state that, to the best of my knowledge, my answers to the questions on this form are complete and correct.

Signature of athlete: _____

Signature of parent or guardian: _____

Date: _____

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FIGURE 2.3

Emergency Information Card

Athlete's name _____ Age _____

Address _____

Primary phone _____ Phone (other) _____

Sport _____

Emergency contacts (include names of two contacts) _____

Parent's or guardian's name _____

Address _____

Primary phone _____ Phone (other) _____

Alternate contact's name _____

Address _____

Primary phone _____ Phone (other) _____

Relationship to athlete _____

Insurance provider _____ Group/policy # _____

Physician's name _____ Phone _____

Are you allergic to any medications? _____ If so, what? _____

Do you have any allergies (e.g., bee stings or dust)? _____

Are you allergic to any foods? _____ If so, what? _____

Do you have ☐ asthma, ☐ diabetes, or ☐ epilepsy? (Check any that apply.)

Do you take any medications? _____ If so, what? _____

Do you wear contact lenses? _____

Other _____

Signature _____ Date _____

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FIGURE 2.4

Facilities Inspection Checklist

Name of inspector _____ Date of inspection _____

Name and location of facility _____

Circle Y (yes) if the facility is in good condition or N (no) if it needs something done to make it acceptable. In the space provided, note what needs to be done.

Gymnasium

Y	N	Inspection item	Notes / action needed
Y	N	Floor (water spots, buckling, loose sections)	
Y	N	Walls (vandalism free and padded, if appropriate)	
Y	N	Lights (all functioning)	
Y	N	Windows (secure)	
Y	N	Roof (adverse impact of weather)	
Y	N	Stairs (well lighted)	
Y	N	Bleachers (support structure sound)	
Y	N	Exits (lights working)	
Y	N	Basketball rims (level, securely attached)	
Y	N	Basketball backboards (no cracks, clean)	
Y	N	Mats (clean, properly stored, no defects)	
Y	N	Uprights or projections (padded)	
Y	N	Wall plugs (covered)	
Y	N	Light switches (all functioning)	
Y	N	Heating or cooling system (temperature control)	
Y	N	Ducts, radiators, and pipes	
Y	N	Thermostats	
Y	N	Fire alarms (regularly checked)	
Y	N	Directions posted for evacuating the gym in case of fire	
Y	N	Fire extinguishers (regularly checked)	
		Other:	

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Locker rooms

Y	N	Inspection item	Notes / action needed
Y	N	Floor	
Y	N	Walls	
Y	N	Lights	
Y	N	Windows	
Y	N	Roof	
Y	N	Showers	
Y	N	Drains	
Y	N	Benches	
Y	N	Lockers	
Y	N	Exits	
Y	N	Water fountains	
Y	N	Toilets	
Y	N	Athletic trainer's room	
		Other:	

Field or outside playing area

Y	N	Inspection item	Notes / action needed
Y	N	Stands	
Y	N	Pitching mound	
Y	N	Dugouts	
Y	N	Track and fences	
Y	N	Sidelines	
Y	N	Sprinklers	
Y	N	Garbage	
Y	N	Security fences	
Y	N	Water fountain	
Y	N	Storage sheds	
Y	N	Goalposts	
Y	N	Net	
Y	N	Net standards	
		Other:	

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Pool area and equipment

Pool

Y	N	Inspection item	Notes / action needed
Y	N	Equipment in good repair	
Y	N	Sanitary	
Y	N	Slipperiness controlled (decks, diving board)	
Y	N	Chemicals safely stored	
Y	N	Regulations and safety rules posted	
		Other:	

Safety line

Y	N	Inspection item	Notes / action needed
Y	N	Located at break point in the pool grade	
Y	N	Bright-colored floats	
Y	N	3/4-inch rope	

Ring buoys

Y	N	Inspection item	Notes / action needed
Y	N	20-inch diameter	
Y	N	50-foot rope length	

Guard chairs

Y	N	Inspection item	Notes / action needed
Y	N	Unobstructed view	
Y	N	Tall enough to see bottom of pool	

Lighting

Y	N	Inspection item	Notes / action needed
Y	N	Adequate visibility	
Y	N	No glare	
Y	N	Penetrates to bottom of pool	
Y	N	Exit light in good repair	
Y	N	Halls and locker rooms meet code requirements	
Y	N	Light switches properly grounded	
Y	N	Emergency generator available	
		Other:	

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Exits

Y	N	Inspection item	Notes / action needed
Y	N	Accessible and secure	
Y	N	Adequate size, number	
Y	N	Self-closing doors	
Y	N	Self-locking doors	
Y	N	Striker plates secure	
Y	N	No obstacles or debris	
Y	N	Office and storage rooms locked	
		Other:	

Track

Y	N	Inspection item	Notes / action needed
Y	N	Throwing circles	
Y	N	Fences	
Y	N	Water fountain	
Y	N	Surface free of debris	
Y	N	Surface free of holes and bumps	
		Other:	

First aid equipment

Y	N	Inspection item	Notes / action needed
Y	N	First aid kit, inventoried and replenished regularly	
Y	N	Stretcher, inventoried and in good repair	
Y	N	Two blankets, inventoried and in good repair	
Y	N	Spine board, inventoried and in good repair	
		Other:	

Recommendations/observations

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Adapted by permission from *American Coaching Effectiveness Program Level 2 Sport Law Workbook* (Champaign, IL: Human Kinetics, 1985), 40-41; J.R. Olson, "Safety Checklists: Making Indoor Areas Hazard-Free," *Athletic Business* (1985): 36-38.

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FIGURE 2.6

Emergency Response Card

Be prepared to give this information to the EMS dispatcher.

No.	Information for EMS dispatcher	Response
1.	Location	
	Street address	
	City or town	
	Zip code	
	Directions (e.g., cross streets or landmarks)	
2.	Phone number from which the call is being made	
3.	Caller's name	
4.	What happened	
5.	How many persons injured	
6.	Condition of injured person(s)	
7.	Help (first aid) being given	

Note: Do not hang up first. Let the EMS dispatcher hang up first.

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FIGURE 2.7

Injury Report

Field	Response
Name of athlete	
Date	_____ Time _____
Person performing initial emergency care	
Mechanism of injury	
Type of injury	
Anatomical areas involved	
First aid administered	
Other treatment administered	
Referral action	
Person performing initial emergency care (signature)	

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